Name: Viktoriya A Pulliam | DOB: 12/6/1989 | MRN: 10132149 | PCP: PCP Pending Apt made | Legal Name: Viktoriya A Pulliam

Initial Visit - Jul 10, 2025

with Daniel Cavalcante, MD at CU Sports Medicine - Boulder

Notes from Care Team

Progress Notes

Morgan C at 7/10/2025 2:30 PM

Viktoriya A Pulliam is here for consult for history of C5-6 fusion with complications

Patient reports that she has had a fusion due to an MVA that she had. The fusion was great for the first year all the tingling went away. But around a year after she would get twitching all over her body and aching like the flu all time. Has had MRI and X-rays. She states that unless she is on 800mg of Ibuprofen and a 1000mg Tylenol together all the time she is in excruciating pain on the right side. Her left arm still spasms but not painful like the right arm. Her right arm is tingling and numb. She is on Lyrica in the evening to sleep but she will still wake up during the night. She states that her previous neurosurgeon said things look the same but she does not think so. She would like to figure out the mechanical symptoms before the Chiarri malformation that she is aware she has. They surgeon that did the surgery is retiring. She wonders about a revision. She is starting to get really worried. She has a child and would love to be able to do things with her without having.

Daniel Cavalcante, MD at 7/10/2025 2:30 PM

Neurosurgery Outpatient Clinic Initial Visit
Daniel Cavalcante M.D.

CC:

Assessment/Plan:

35 y.o. female with a prior history of trauma with spinal cord injury. She has a spinal cord signal at C5-C6, likely myelomalacia.
The MRI of the cervical spine shows moderate adjacent level degeneration at C4-C5 with mild right-sided foraminal stenosis.
In the physical exam she has good strength in all 4 extremities and normal reflexes.
Patient has a myelomalacia in the cervical spine that can be the cause of her symptoms.
I do not recommend further surgical intervention at the moment. I do not see any significant ongoing spinal stenosis.
I am referring the patient to another neurologist for second opinion.
I also discussed with the patient the possibility of doing a C4-C5 injection to evaluate if her symptoms can be coming from there, although I believe it is more likely to be if from the prior spinal cord injury.
Patient will discuss with her pain management doctor the possibility of injecting the cervical spine for diagnostic and therapeutic purposes.
Patient has mild low position of the cerebellar tonsils but she has no significant symptoms suggestive of Chiari I malformation.

HPI:

Viktoriya A Pulliam is a 35 y.o. female referred by for neurosurgical consultation regarding
It was a pleasure to see Ms Pulliam in clinic today.

Patient has a history of car accidents in the past with a severe spinal cord injury at the time. Patient underwent a C5-C7 ACDF at the time. She did improve from her symptoms for about a year and since then she has been declining with constant cervical pain. Most recently she developed a numbness throughout the right upper extremity. She denies clumsiness, dexterity issues, balance issues.
She denies any significant weakness.
She reports she was evaluated by neurology to rule out multiple sclerosis.
Patient denies any bowel or bladder problems. Patient denies smoking cigarettes.
Patient denies exertional headaches

Medications and Allergies

Allergies as of 07/10/2025
- (Not on File)

Current Medications
No current outpatient medications on file.

Past Medical History

Past Medical History
No past medical history on file.

Past Surgical History
No past surgical history on file.

Social History:

Collapsed by default

Family Medical History:

Family History
No family history on file.

Review of Systems:

Neurological ROS as per HPI

10 point review of systems completed.
All other systems reviewed are negative.

Objective:

Physical Exam:

Vital Signs: Reviewed

General:
-Well developed and well nourished. No acute distress. Cooperative with the exam.

Mental Status:
-Awake and oriented to person, place, and time. Affect is normal.
-Fund of knowledge appropriate
-Recent and remote memory are intact
-Attention span and concentration appear normal.
-Language function is normal. There is no evidence of aphasia in conversational speech.

Cranial nerves:

-CN III, IV, VI: Pupils equal, round; extraocular movements intact, no ptosis, no nystagmus
-CN V: Facial sensation intact in V1 through V3 distributions
-CN VII: Face symmetric
-CN VIII: Hearing intact to conversational speech
-CN IX, X: Palate elevates symmetrically; normal phonation
-CN XI: Symmetric full strength of sternocleidomastoid and trapezius muscles
-CN XII: Tongue protrudes midline

Motor:

-Strength: 5/5 throughout

Reflexes:
-1-2+ DTR's throughout

Coordination:
-Intact to FTN
-Intact to HTS
-No tremors

Gait:
-Stable
-Station normal

Data: as per HPI

I personally spent a total of 60 minutes today preparing to see the patient, on direct patient care, independently interpreting results, documenting, and care coordination. My assessment is is detailed above.

Daniel Cavalcante, M.D.,
University of Colorado Department of Neurosurgery

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07/10/2025 - Ancillary Procedure in CU RAD Diagnostic - Boulder
FACESHEET

CU CENTRAL
CU RAD DIAGNOSTIC - BOULDER
2150 STADIUM DR
BOULDER CO 80309
Dept: 303-315-9900
Dept Fax: 303-315-9902

PATIENT INFORMATION:

Patient Name:	**Pulliam, Viktoriya A**	**Patient DOB:**	12/6/1989 (35 yrs)
Patient MRN:	**10132149**	**Legal Sex:**	Female
Preferred Name:		Gender Identity:	Female
Patient Address:		Patient Temporary Address: (if applicable)	
Patient Phone (H):		Email Address:	
Patient Phone (W):		Patient Language:	English
Patient Phone (M):		Needs Interpreter?:	
Marital Status:	Married	Religion:	

ENCOUNTER/ADMISSION INFORMATION:

CSN:	426225341	Unit:	
HAR:		Room:	
Visit Date:	7/10/2025 2:55 PM	Bed:	
Attending Physician:		Patient Class:	Outpatient
Admitting Physician:		Service:	
PCP:	Ahmed Sayed Ahmed, MD	Admission Type:	Elective
Referring Provider:	Cavalcante, Daniel Maldonado, MD	Admission Source:	
Referring Provider Phone:	720-718-8288	Appointment Dept:	CU RAD DIAG BOU*
Referring Provider Fax:	720-718-0969	Appointment Prov:	CU BOULDR DIAG ROOM 1 [6*
Alt Referring Provider:		Visit Type:	CU BOULDR DIAG EXAM
Alt Referring Prov Phone:		Diagnosis:	History of fusion of cer*
Alt Referring Prov Fax:		Procedure:	XR CERVICAL SPINE (AP,LA*

EMERGENCY CONTACT INFORMATION:

Emergency Contact:	PULLIAM,BRYAN	Phone (H):	
Relation to Patient:	Spouse	Phone (M):	
Language:		Legal Guardian?:	

GUARANTOR INFORMATION:

Guarantor Name:	PULLIAM,VIKTORIYA A	Guarantor Type:	Personal/Family
Guarantor ID:	110801727	Relation to Patient:	Self
Guarantor Address:	Gillette, WY 82716-4822	Guar Phone (H):	
		Guar Phone (W):	
		Guar Phone (M):	

INSURANCE INFORMATION:

	PRIMARY	SECONDARY	TERTIARY
Payor:			
Plan:			
Group Number:			
Insurance Type:			
Subscriber ID:			

07/10/2025 - Ancillary Procedure in CU RAD Diagnostic - Boulder FACESHEET (continued)

Subscriber Name:			
Rel to Patient:			

07/10/2025 - Ancillary Procedure in CU RAD Diagnostic - Boulder (continued)

Reason for Visit

Visit Diagnoses *[last edited by SER U on 7/10/2025 1453]*

- **(primary)**
- History of fusion of cervical spine

Visit Information

Provider Information

Referring Provider

Daniel Cavalcante, MD

Department

Name	Address	Phone	Fax
CU RAD Diagnostic - Boulder	2150 Stadium Dr Boulder CO 80309	303-315-9900	303-315-9902

Medication List

Medication List

ⓘ This report is for documentation purposes only. The patient should not follow medication instructions within. For accurate instructions regarding medications, the patient should instead consult their physician or after visit summary.

Active at the End of Visit

None

Stopped in Visit

None

Imaging Results

Imaging

XR CERVICAL SPINE (AP,LAT,FLEX,EXT) (Final result)

XR CERVICAL SPINE (AP,LAT,FLEX,EXT) Resulted: 07/11/25 0857, Result status: Final result

Ordering provider: Daniel Cavalcante, MD 07/10/25 1453
Resulted by: Mary Lowry, MD
Performed: 07/10/25 1453 - 07/10/25 1533
Resulting lab: POWERSCRIBE
Acknowledged by: Michelle Degrave, PA-C on 07/14/25 1535

Order status: Completed
Filed by: Rad E 07/11/25 0900
Accession number: 43412698

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
33 - PWS	POWERSCRIBE	Unknown	Unknown	09/14/11 2208 - Present

Study Result

naging Results (continued)

XR CERVICAL SPINE (AP,LAT,FLEX,EXT) 7/10/2025 2:53 PM

INDICATION: cervical pain

COMPARISON: None

FINDINGS:
C5-C7 anterior cervical discectomy and fusion without complication. No dynamic listhesis. Normal prevertebral soft tissues and lung apices.

IMPRESSION:
C5-C7 anterior cervical discectomy and fusion without complication.

If you are a healthcare provider and have any questions regarding this or any musculoskeletal radiology report please call: 720-848-3435. The musculoskeletal radiology reading area location is: 4th floor AOP. We are staffed 8 AM to 5 PM, Monday through Friday.

If you are a patient and have questions about your report, please contact your healthcare provider.

Final Report E-Signed By: MK Jesse Lowry, MD at 7/11/2025 8:57 AM
WSN:RAD-D-C7MLZC3

Indications

History of fusion of cervical spine [Z98.1 (ICD-10-CM)]

All Reviewers List

Michelle Degrave, PA-C on 7/14/2025 15:35

07/10/2025 - Ancillary Procedure in CU RAD Diagnostic - Boulder (continued)

Case 8109783

Surgery Information

General Information

Date: 7/10/2025	Time:	Status: Posted
Location: ZZANESTHESIA - DO NOT SCHEDULE	Room:	Service:
Patient class:	Case classification:	

Diagnosis Information

No post-op diagnosis codes associated with the log.

Clinical Documentation

Case Tracking Events

Event	Time In
In Facility	1453

Event Tracking

No event timings documented

Patient Preparation

None

Counts by Panel

Panel 1

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder
FACESHEET

CU SPORTS MED BOULDER
CU SPORTS MEDICINE - BOULDER
2150 STADIUM DR FL 2
BOULDER CO 80309
Dept: 303-315-9900
Dept Fax: 303-315-9902

PATIENT INFORMATION:

Patient Name:	**Pulliam, Viktoriya A**	**Patient DOB:**	12/6/1989 (35 yrs)
Patient MRN:	**10132149**	**Legal Sex:**	Female
Preferred Name:		Gender Identity:	Female
Patient Address:	100 W Hogeye Dr Gillette WY 82716-4822	Patient Temporary Address: (if applicable)	
Patient Phone (H):		Email Address:	[illegible]om
Patient Phone (W):		Patient Language:	English
Patient Phone (M):	[illegible]	Needs Interpreter?:	
Marital Status:	Married	Religion:	[illegible]

ENCOUNTER/ADMISSION INFORMATION:

CSN:	424838465	Unit:	
HAR:		Room:	
Visit Date:	7/10/2025 2:30 PM	Bed:	
Attending Physician:		Patient Class:	
Admitting Physician:		Service:	
PCP:	Ahmed Sayed Ahmed, MD	Admission Type:	Elective
Referring Provider:	System, Provider Not In	Admission Source:	
Referring Provider Phone:	N/A	Appointment Dept:	CU SPORTS MED B*
Referring Provider Fax:		Appointment Prov:	DANIEL MALDONADO CAVALCA*
Alt Referring Provider:		Visit Type:	NEW PATIENT
Alt Referring Prov Phone:		Diagnosis:	Arnold-Chiari malformati*
Alt Referring Prov Fax:		Procedure:	AMB REF TO NEUROLOGY [RE*

EMERGENCY CONTACT INFORMATION:

Emergency Contact:	PULLIAM,BRYAN	Phone (H):	
Relation to Patient:	Spouse	Phone (M):	[illegible]
Language:		Legal Guardian?:	

GUARANTOR INFORMATION:

Guarantor Name:	PULLIAM,VIKTORIYA A	Guarantor Type:	Personal/Family
Guarantor ID:	110801727	Relation to Patient:	Self
Guarantor Address:	Gillette, WY 82716-4822	Guar Phone (H):	
		Guar Phone (W):	
		Guar Phone (M):	

INSURANCE INFORMATION:

	PRIMARY	SECONDARY	TERTIARY
Payor:			
Plan:	[illegible]		
Group Number:			
Insurance Type:			
Subscriber ID:	[illegible]		

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder FACESHEET (continued)

Subscriber Name:			
Rel to Patient:			

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder (continued)

Reason for Visit

Chief Complaint *[last edited by Morgan C, MA on 7/10/2025 1450]*

- New Patient Consult (consult for history of C5-6 fusion with complications)

Visit Diagnosis *[last edited by Annie M, MA on 7/9/2025 1634]*

- **History of fusion of cervical spine (primary)**

Visit Information

Provider Information

Encounter Provider	Authorizing Provider	Referring Provider
Daniel Cavalcante, MD	Daniel Cavalcante, MD	Provider S

Department

Name	Address	Phone	Fax
CU Sports Medicine - Boulder	2150 Stadium Dr Fl 2 Boulder CO 80309	303-315-9900	303-315-9902

Level of Service

Level of Service	Modifiers
PR OFFICE/OUTPATIENT NEW HIGH MDM 60 MINUTES	CU MED ONLY BBT BILL BASED ON TIME

Log History

LOS History

Referral

Consultation #51541123 *[last edited by User R on 7/13/2025 1628]*

Reason: Specialty Services Required
Class: Incoming
Status updated on: 6/23/2025
Priority: Routine
Status: Authorized - Auto No Authorization Needed
Valid dates: From 6/23/2025 to 6/22/2026

Referred From

Location: UCH OUTPATIENT SERVICES
Provider: Provider S
Department: NON-UCH
Provider address: University of Colorado

Referred To

Department: AMC NEUROLOGY OP
Specialty: Neurology
Department phone: 720-848-2080

Visits

Requested: 12 Authorized: 12 Completed: 1 Scheduled: 0

Procedures

REF46 - AMBULATORY REFERRAL TO NEUROLOGY

Number requested: 1
Number approved: 1

Diagnoses

- Q07.00 (ICD-10-CM) - Arnold-Chiari malformation (HC CODE)

Referral (continued)

- R41.89 (ICD-10-CM) - Other symptoms and signs involving cognitive functions and awareness
- M62.81 (ICD-10-CM) - Muscle weakness

Referral Notes

General by Diana M at 6/26/2025 0944

Summary: Spine fusion 2nd opinion

Patient has chiari malformation however she is trying to get a second opinion for the cervical fusion she had recently. Scheduled with spine for 2nd opinion neck surgery.

Provider Comments by Veronica P at 6/23/2025 1134

Summary: Provider Comments

Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Order

AMBULATORY REFERRAL TO NEUROLOGY [1038159859]

Electronically signed by: **Veronica P on 06/23/25 1134** Status: **Active**
This order may be acted on in another encounter.
Ordering user: Veronica P 06/23/25 1134 Ordering provider: Provider S
Authorized by: Provider S Ordered during: Orders Only on 06/23/2025
Diagnoses
Arnold-Chiari malformation (HC CODE) [Q07.00]
Other symptoms and signs involving cognitive functions and awareness [R41.89]
Muscle weakness [M62.81]
Order comments: Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Triage

Triage Information

Decision: None Schedule by date: 7/23/2025

Coverages

AETNA

Plan: AETNA POS	Covered: Covered	From: 8/31/2018	Member #: W245811436

AETNA

Plan: AETNA POS	Covered: Covered	From: 8/31/2018	Member #: W245811436

Physician Progress Notes

Progress Notes

Daniel Cavalcante, MD at 7/10/2025 1430

Author: Daniel Cavalcante, MD Service: — Author Type: Physician
Filed: 7/10/2025 5:00 PM Encounter Date: 7/10/2025 Status: Signed
Editor: Daniel Cavalcante, MD (Physician)

Neurosurgery Outpatient Clinic Initial Visit
Daniel Cavalcante M.D.

CC:

Assessment/Plan:
35 y.o. female with a prior history of trauma with spinal cord injury. She has a spinal cord signal at C5-C6, likely myelomalacia.
The MRI of the cervical spine shows moderate adjacent level degeneration at C4-C5 with mild right-sided foraminal stenosis.
In the physical exam she has good strength in all 4 extremities and normal reflexes.
Patient has a myelomalacia in the cervical spine that can be the cause of her symptoms.
I do not recommend further surgical intervention at the moment. I do not see any significant ongoing spinal stenosis.
I am referring the patient to another neurologist for second opinion.
I also discussed with the patient the possibility of doing a C4-C5 injection to evaluate if her symptoms can be coming from there, although I believe it is more likely to be if from the prior spinal cord injury.
Patient will discuss with her pain management doctor the possibility of injecting the cervical spine for diagnostic and therapeutic purposes.
Patient has mild low position of the cerebellar tonsils but she has no significant symptoms suggestive of Chiari I malformation.

HPI:
Viktoriya A Pulliam is a 35 y.o. female referred by for neurosurgical consultation regarding
It was a pleasure to see Ms Pulliam in clinic today.
Patient has a history of car accidents in the past with a severe spinal cord injury at the time. Patient underwent a C5-C7 ACDF at the time. She did improve from her symptoms for about a year and since then she has been declining with constant cervical pain. Most recently she developed a numbness throughout the right upper extremity. She denies clumsiness, dexterity issues, balance issues.
She denies any significant weakness.
She reports she was evaluated by neurology to rule out multiple sclerosis.
Patient denies any bowel or bladder problems. Patient denies smoking cigarettes.
Patient denies exertional headaches

Medications and Allergies

Allergies as of 07/10/2025
- (Not on File)

Current Medications
No current outpatient medications on file.

Past Medical History
Past Medical History
No past medical history on file.

Past Surgical History
No past surgical history on file.

Social History:
Collapsed by default

Family Medical History:
Family History
No family history on file.

Review of Systems:
Neurological ROS as per HPI

10 point review of systems completed.
All other systems reviewed are negative.

Objective:
Physical Exam:

Vital Signs: Reviewed

General:
-Well developed and well nourished. No acute distress. Cooperative with the exam.

Mental Status:
-Awake and oriented to person, place, and time. Affect is normal.
-Fund of knowledge appropriate
-Recent and remote memory are intact
-Attention span and concentration appear normal.
-Language function is normal. There is no evidence of aphasia in conversational speech.

Cranial nerves:

-CN III, IV, VI: Pupils equal, round; extraocular movements intact, no ptosis, no nystagmus
-CN V: Facial sensation intact in V1 through V3 distributions
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-CN IX, X: Palate elevates symmetrically; normal phonation
-CN XI: Symmetric full strength of sternocleidomastoid and trapezius muscles
-CN XII: Tongue protrudes midline

Motor:

-Strength: 5/5 throughout

Reflexes:
-1-2+ DTR's throughout

Coordination:
-Intact to FTN
-Intact to HTS
-No tremors

Gait:
-Stable
-Station normal

Data: as per HPI

I personally spent a total of 60 minutes today preparing to see the patient, on direct patient care, independently interpreting results, documenting, and care coordination. My assessment is is detailed above.

Daniel Cavalcante, M.D.,
University of Colorado Department of Neurosurgery

Electronically signed by Daniel Cavalcante, MD at 7/10/2025 5:00 PM

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder (continued)

ll Other Notes

Progress Notes by Morgan C, MA at 7/10/2025 1430

Viktoriya A Pulliam is here for consult for history of C5-6 fusion with complications

Patient reports that she has had a fusion due to an MVA that she had. The fusion was great for the first year all the tingling went away. But around a year after she would get twitching all over her body and aching like the flu all time. Has had MRI and X-rays. She states that unless she is on 800mg of Ibuprofen and a 1000mg Tylenol together all the time she is in excruciating pain on the right side. Her left arm still spasms but not painful like the right arm. Her right arm is tingling and numb. She is on Lyrica in the evening to sleep but she will still wake up during the night. She states that her previous neurosurgeon said things look the same but she does not think so. She would like to figure out the mechanical symptoms before the Chiarri malformation that she is aware she has. They surgeon that did the surgery is retiring. She wonders about a revision. She is starting to get really worried. She has a child and would love to be able to do things with her without having.

Electronically signed by Morgan C, MA at 7/10/2025 5:00 PM

Medication List

Medication List

ⓘ This report is for documentation purposes only. The patient should not follow medication instructions within. For accurate instructions regarding medications, the patient should instead consult their physician or after visit summary.

Active at the End of Visit

None

Stopped in Visit

None

Other Orders

Outpatient Referral

AMBULATORY REFERRAL TO NEUROLOGY (Active)

Electronically signed by: **Veronica P on 06/23/25 1134** Status: **Active**
This order may be acted on in another encounter.
Ordering user: Veronica P 06/23/25 1134 Ordering provider: Provider S
Authorized by: Provider S Ordering mode: Referral Order
Ordered during: Orders Only on 06/23/2025
Frequency: Routine 06/23/25 - Class: Incoming Referral
Quantity: 1
Diagnoses
Arnold-Chiari malformation (HC CODE) [Q07.00]
Other symptoms and signs involving cognitive functions and awareness [R41.89]
Muscle weakness [M62.81]

Questionnaire

Question	Answer
My Clinical Question:	CHIARI MALFORMATION, NEUROMUSCULAR DISORDERS

Order comments: Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Referral Details

Referred By		Referred To	Type	Priority
Provider S University of Colorado	Diagnoses: Arnold-Chiari malformation (HC CODE) Other symptoms and signs			

ther Orders (continued)

	involving cognitive functions and awareness Muscle weakness Order: Ambulatory Referral To Neurology Reason: Specialty Services Required	**Amc Neurology Op** 1635 Aurora Ct Aurora CO 80045-2541 Phone: 720-848-2080 Fax: 720-848-2106 Specialty: Neurology	Consultation	Routine

Comment: Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

AMB REF TO NEUROLOGY (Active)

Electronically signed by: **Daniel Cavalcante, MD on 07/10/25 1700** Status: **Active**
Mode: Ordering in Per Protocol or Collaborative Practice Orders (Co-sign Req'd) mode
Communicated by: Morgan C, MA
Ordering user: Morgan C, MA 07/10/25 1611
Ordering provider: Daniel Cavalcante, MD
Authorized by: Daniel Cavalcante, MD
Ordering mode: Per Protocol or Collaborative Practice Orders (Co-sign Req'd)
Frequency: Routine 07/10/25 -
Class: Internal Referral
Quantity: 1
Diagnoses
History of fusion of cervical spine [Z98.1]

Questionnaire

Question	Answer
Referred for:	Other
My Clinical Question:	worsening symptoms of numbness eval for second opinion on spinal cord injury

Order comments: Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Referral Details

Referred By		Referred To	Type	Priority
Daniel Cavalcante, MD 1551 Professional Ln Ste 200 Longmont CO 80501 Phone: 720-718-8288 Fax: 720-718-0969	Diagnoses: History of fusion of cervical spine Order: Amb Ref To Neurology Reason: Specialty Services Required	**Amc Neurology Op** 1635 Aurora Ct Aurora CO 80045-2541 Phone: 720-848-2080 Fax: 720-848-2106 Specialty: Neurology	Consultation	Routine

Comment: Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Flowsheets

Anthropometrics

Row Name	07/10/25 1452
Anthropometrics	
Weight Change	0 -MC at 07/10/25 1453

Custom Formula Data

Row Name	07/11/25 0334	07/10/25 1452	07/02/25 1426
OTHER			
PDMP Narcotics Score Ones Value	0 -IE at 07/11/25 0334	—	0 -IE at 07/02/25 1426
PDMP Narcotics Score Tens Value	6 -IE at 07/11/25 0334	—	6 -IE at 07/02/25 1426

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder (continued)

owsheets (continued)

PDMP Narcotics Score Hundreds Value	1 -IE at 07/11/25 0334	—	1 -IE at 07/02/25 1426
PDMP Sedatives Score Ones Value	1 -IE at 07/11/25 0334	—	1 -IE at 07/02/25 1426
PDMP Sedatives Score Tens Value	3 -IE at 07/11/25 0334	—	3 -IE at 07/02/25 1426
PDMP Sedatives Score Hundreds Value	1 -IE at 07/11/25 0334	—	1 -IE at 07/02/25 1426
PDMP Stimulants Score Ones Value	0 -IE at 07/11/25 0334	—	0 -IE at 07/02/25 1426
PDMP Stimulants Score Tens Value	0 -IE at 07/11/25 0334	—	0 -IE at 07/02/25 1426
PDMP Stimulants Score Hundreds Value	0 -IE at 07/11/25 0334	—	0 -IE at 07/02/25 1426
PDMP Total Risk Score Ones Value	0 -IE at 07/11/25 0334	—	0 -IE at 07/02/25 1426
PDMP Total Risk Score Tens Value	2 -IE at 07/11/25 0334	—	2 -IE at 07/02/25 1426
PDMP Total Risk Score Hundreds Value	3 -IE at 07/11/25 0334	—	3 -IE at 07/02/25 1426
Target index of 2.0	—	3.6 -MC at 07/10/25 1453	—
Target index of 2.2	—	4.05 -MC at 07/10/25 1453	—
Target index of 2.5	—	4.5 -MC at 07/10/25 1453	—
Bolus 30ml/IBW kg	—	1710 ml -MC at 07/10/25 1453	—
Bolus 30ml/kg	—	2108.84 ml -MC at 07/10/25 1453	—
BMI (Calculated)	—	25.79 -MC at 07/10/25 1453	—
Weight gain/loss (kg)	—	70.31 kg -MC at 07/10/25 1453	—
IBW (lbs) - Calculated	—	125 lb -MC at 07/10/25 1453	—
Today's Excess Weight (lbs)	—	30 -MC at 07/10/25 1453	—
IBW (lbs) - Based on BMI 25	—	150.25 -MC at 07/10/25 1453	—
Today's Excess Weight (lbs) BMI 25	—	4.75 -MC at 07/10/25 1453	—
IBW/kg (Calculated)	—	57 kg -MC at 07/10/25 1453	—
% Weight Change Since Birth	—	0 % -MC at 07/10/25 1453	—
IBW/kg (Calculated)	—	57 kg -MC at 07/10/25 1453	—
Current BSA	—	1.8 -MC at 07/10/25 1453	—
Weight gain/loss	—	70308 gms -MC at 07/10/25 1453	—
Low: Female	—	2109.24 -MC at	—

07/10/2025 - Initial Visit in CU Sports Medicine - Boulder (continued)

lowsheets (continued)

Calculated Energy for BMI < 25		07/10/25 1453	
High: Female Calculated Energy for BMI < 25	—	86.8 -MC at 07/10/25 1453	—
Female: BMI > 25	—	1398.95 -MC at 07/10/25 1453	—
Weight gain/loss (kg)	—	70.31 kg -MC at 07/10/25 1453	—
Grams Protein Needed (calculated)	—	70.31 -MC at 07/10/25 1453	—
Height and Weight			
BSA (Calculated - sq m)	—	1.8 sq meters -MC at 07/10/25 1453	—
Weight/Height Bariatric Measurements			
Weight in Pounds (lbs)	—	155 lb -MC at 07/10/25 1453	—
Ideal Body Weight	—	125 lb -MC at 07/10/25 1453	—
Body Composition Result			
Body Mass (lbs)	—	155 -MC at 07/10/25 1453	—

PDMP NARxCHECK Scores

Row Name	07/11/25 0334	07/02/25 1426
PDMP NARxCHECK Scores		
Narcotics Score	160 -IE at 07/11/25 0334	160 -IE at 07/02/25 1426
Sedatives Score	131 -IE at 07/11/25 0334	131 -IE at 07/02/25 1426
Stimulants Score	000 -IE at 07/11/25 0334	000 -IE at 07/02/25 1426
Overdose Risk Score	320 -IE at 07/11/25 0334	320 -IE at 07/02/25 1426

Vitals/Pain

Row Name	07/10/25 1452
Height and Weight	
Height	1.651 m (5' 5") -MC at 07/10/25 1453
Weight	70.3 kg (155 lb) -MC at 07/10/25 1453
BMI (Calculated)	25.79 -MC at 07/10/25 1453

User Key

(r) = Recorded By, (t) = Taken By, (c) = Cosigned By

Initials	Name	Provider Type	Discipline
MC	Morgan C, MA	Medical Assistant	—
IE	Interface E	—	—

After Visit Summary

After Visit Summary (below)

AFTER VISIT SUMMARY

Sports Medicine
UNIVERSITY OF COLORADO

Viktoriya A. Pulliam DoB: 12/6/1989 7/10/2025 2:30 PM CU Sports Medicine - Boulder 303-315-9900

Instructions from Daniel Cavalcante, MD

Referral to Neurology
Where: **UCHealth Neurology** Clinic - **Anschutz** Medical Campus
Address: **1635 Aurora Ct** Aurora CO **80045-2541**
Phone: 720-848-2080
Multiple visits requested (expires 7/9/2026)
Referrals for Dizziness/Vertigo: If no other neurologic symptoms or diagnosis, place ENT consult or refer back to PCP.

Today's Visit

You saw Daniel Cavalcante, MD on Thursday July 10, 2025. The following issue was addressed: History of fusion of cervical spine.

BMI	Weight	Height
25.79	155 lb	5' 5"

What's Next

You currently have no upcoming appointments scheduled.

Disclaimer

Information Sharing

We believe in information transparency, and that you deserve to see your health information as soon as it is available.

This means you could see some information before we have a chance to review it. It might take a few days to review routine test results and explain what they mean for you, but we will get in touch with you quickly if the results are urgent.

MHC

My Health Connection

View your After Visit Summary, schedule appointments, send messages, and refill medications by downloading the UCHealth Mobile App for Apple or Android.

Download the UCHealth Mobile App to create an account today or register online at:
https://www.uchealth.org/access-my-health-connection/

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Your Medication List as of July 10, 2025 5:00 PM

You have not been prescribed any medications.

Radiology Contact Information

Please call 1.844.RAD.APPT or 1.844.723.2778 to schedule radiology tests, including, but not limited to:

- Cat Scan
- Nuclear Medicine
- Ultrasound
- MRI
- Dexa (Bone Density Scan)
- PET
- X-Ray
- Mammography and other breast imaging

Our hours of operation are 7:00 a.m. to 5:30 p.m.

Thank you for allowing us to care for you!

Things to know about the COVID-19 vaccine:

- General scheduling is open to anyone age 6 months or older. We will be adding new appointment times every day, so if you don't find times that work for you, please check back the next day.
- For patients recently diagnosed with COVID-19, vaccination should wait until the person has recovered from their symptoms and they have completed isolation.
- **Caution:** Patients with a history of immediate allergic reaction to polysorbate or to a previous dose of COVID-19 vaccine or any of its components including polyethylene gycol (PEG) SHOULD NOT get the COVID-19 vaccine. If you have any questions about your history of allergic reactions, please talk to your doctor.

Here is your list of medicines. If you have questions about a medicine, ask the provider who prescribed it. It is important to throw out your old medicine lists. You should keep your newest medication list with you in case of emergency.

Allergies

Naproxen	UNKNOWN (SEE COMMENTS), ARRHYTHMIA
Penicillins	Other Reaction, Rash, UNKNOWN (SEE COMMENTS)

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Health Tips

1. We recommend writing down your medical questions and your treatment goals for your next visit.

2. Unless you and your doctor have agreed on a different goal, we suggest an hour of physical activity every day for healthy adults. This is recommended by the American Heart Association.

Tips for Healthy Living

- Limit high-calorie snacks. Instead, choose lower calorie, healthy snacks, such as:
  - a carrot with a low-fat dip
  - a few whole-wheat crackers with a teaspoon of peanut butter
- Skip or share desserts with sugar and that are high in fat. Instead, eat a piece of fresh fruit or add cut-up fruit to low-fat, plain yogurt.
- Cut back on high-calorie drinks. If you drink alcohol, limit the amount you drink. Drink water instead of soft drinks. Instead of drinking a jumbo-sized juice, drink a small glass of 100% fruit juice or eat a piece of fresh fruit.
- Eat smaller amounts. Use a cup to measure your food. Many people are surprised to learn they are eating much more than they think until they measure their food!
- Research supports that 30 to 60 minutes of physical activity on 5 or more days of the week lowers risk of heart disease, stroke, high blood pressure, and type 2 diabetes.
- Pick up and use the CalorieKing™ book from your local bookstore to help you make better choices. You can also use the website www.calorieking.com or the app.

Schedule your next appointment online. Download the UCHealth Mobile App or login to My Health Connection on uchealth.org.

Suicide and Crisis Line

If you or your loved one are having thoughts of suicide or self-harm, get or call for help immediately. These contacts are open at all times (24/7/365) to provide confidential help for any mental health, substance use, or emotional crisis.

- **Colorado Crisis Line**
  - **Call: 1-844-493-TALK (8255) or**
  - **Text: TALK to 38255 or**
  - **Visit the website: http://coloradocrisisservices.org/chat/**
- **988 Suicide and Crisis Lifeline**
  - **Call or Text: 988 or**
  - **Visit the website: https://988lifeline.org**

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07/10/2025 - Initial Visit in CU Sports Medicine - Boulder (continued)

Documents (continued)

Health History Questionnaire - Scan on 7/15/2025 8:10 AM: CU SPORTS MED - Pain Diagram

Scan (below)

Sports Medicine & Performance Center
UNIVERSITY OF COLORADO
IN PARTNERSHIP WITH BOULDER COMMUNITY HEALTH

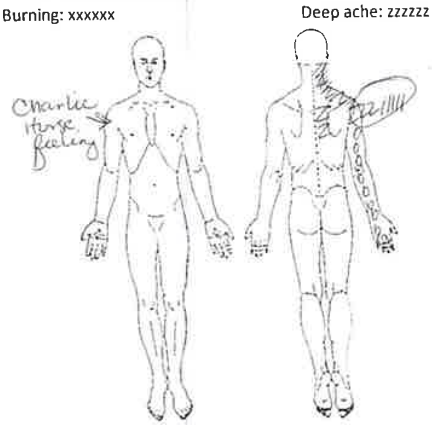
MRN: 10132149 CSN:424838465
Pulliam, Viktoriya A
Svc Date: 7/10/25
DOB: 12/6/1989 35yrs
Prv: CAVALCANTE, DANI*

PAIN DIAGRAM AND PAIN RATING

Please use the diagram below to indicate the symptoms you have experienced over the past 24 hours. Use the key to indicate the type of symptoms.

KEY:

Pins and needles: 000000
Burning: xxxxxx
Stabbing: //////
Deep ache: zzzzzz



Please rate your current level of pain on the following scale (*circle one*).

0 1 2 3 4 5 6 7 8 9 10
no pain — worst imaginable pain

1000mg Tylenol
800mg Advil

Please rate your worst level of pain in the last 24 hours on the following scale (*circle one*).

0 1 2 3 4 5 6 7 8 9 10
no pain — worst imaginable pain

Please rate your best level of pain in the last 24 hours on the following scale (*circle one*).

0 1 2 3 4 5 6 7 8 9 10
no pain — worst imaginable pain

ocuments (continued)

Medicine

AMOUNT NOW DUE
$0.00

Your Detail of Services

PATIENT NAME	DATE	DESCRIPTION	CHARGE AMOUNT
VIKTORIYA A PULLIAM	***Invoice #: 46926616 Provider: MARY K LOWRY MD***		
	07/10/2025	(72050) X-RAY SPINE CERVICAL 4 OR 5 VIEW	$64.00
		Total Charges:	$64.00
		A T	[illegible]
		CREDIT CARD PMT ON ACCT	[illegible]
		Diagnosis Code: M54.2	
		Invoice Balance	0.00
VIKTORIYA A PULLIAM	***Invoice #: 47036124 Provider: DANIEL M CAVALCANTE MD***		
	07/10/2025	(99205) OUTPT NEW VST-LVL V	$872.00
		Total Charges:	$872.00
			1
			19
		[illegible] CARD P[illegible]	[illegible]
		Diagnosis Code: Z98.1	
		Invoice Balance	
		Amount You Owe Now:	

Billing questions or an itemized bill request?

Call our Patient Services department at 303-493-7700 8:00am to 5:00pm MST.